

Medical Devices English Jargon Quick Reference

Field-specific terms, contrast pairs, and high-pressure sentence frames

Audience: medical device engineers, product managers, quality and regulatory staff, clinical specialists, manufacturing teams, field-service teams, and device-company leaders

Focus: A device-focused professional English curriculum for design controls, risk management, usability, verification and validation, regulatory pathways, complaint handling, quality systems, manufacturing, and clinical-user dialogue.

Designed for advanced ESL learners who already use professional English and need industry-specific terminology, realistic meetings, role-play pressure, careful pushback, and polished workplace outputs.

Teaching stance: this is language and workplace-communication training, not legal, medical, financial, safety, or regulatory advice. Instructors should connect every scenario to the learner's current company policies, local rules, and approved procedures.

Nomenclature and Jargon

These are classroom working definitions. Learners should adapt wording to their organization's policies, systems, and local regulatory environment.

User Needs and Design Inputs

Term	Working meaning	Collocations	Contrast and workplace line
user need	In medical devices, user need is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to translate user pain points into controlled requirements.	clarify user need; document user need	Contrast: user need should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify user need and the evidence that supports it.
design input	In medical devices, design input is a controlled description of how a product, service, system, or built asset is intended to work. Use it when teams need to translate user pain points into controlled requirements.	clarify design input; document design input	Contrast: design input should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify design input and the evidence that supports it.
intended use	In medical devices, intended use is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to translate user pain points into controlled requirements.	clarify intended use; document intended use	Contrast: intended use should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify intended use and the evidence that supports it.
traceability	In medical devices, traceability is information that can be checked to support a claim, decision, investigation, or compliance conclusion. Use it when teams need to translate user pain points into controlled requirements.	clarify traceability; document traceability	Contrast: traceability should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify traceability and the evidence that supports it.

Risk Management and Hazard Analysis

Term	Working meaning	Collocations	Contrast and workplace line
hazard	In medical devices, hazard is a condition or event that can create harm, loss, noncompliance, delay, or unacceptable performance. Use it when teams need to discuss hazards, harms, mitigations, and residual risk.	assess hazard; mitigate hazard	Contrast: hazard identifies a possible or current exposure; it is not the same as accepting that exposure. Example: We need to assess hazard, name the owner, and agree on the trigger for escalation.
harm	In medical devices, harm is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to discuss hazards, harms, mitigations, and residual risk.	clarify harm; document harm	Contrast: harm should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify harm and the evidence that supports it.
FMEA	Failure mode and effects analysis, a structured method for identifying how a process or product can fail and prioritizing controls.	clarify FMEA; document FMEA	Contrast: FMEA should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify FMEA and the evidence that supports it.

Term	Working meaning	Collocations	Contrast and workplace line
residual risk	In medical devices, residual risk is a condition or event that can create harm, loss, noncompliance, delay, or unacceptable performance. Use it when teams need to discuss hazards, harms, mitigations, and residual risk.	assess residual risk; mitigate residual risk	Contrast: residual risk identifies a possible or current exposure; it is not the same as accepting that exposure. Example: We need to assess residual risk, name the owner, and agree on the trigger for escalation.

Verification, Validation, and Design Review

Term	Working meaning	Collocations	Contrast and workplace line
verification	In medical devices, verification is a repeatable control mechanism used to prevent, detect, or confirm conditions before work proceeds. Use it when teams need to distinguish building the product right from building the right product.	clarify verification; document verification	Contrast: verification should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify verification and the evidence that supports it.
validation	In medical devices, validation is a repeatable control mechanism used to prevent, detect, or confirm conditions before work proceeds. Use it when teams need to distinguish building the product right from building the right product.	clarify validation; document validation	Contrast: validation should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify validation and the evidence that supports it.
design review	In medical devices, design review is a structured examination of evidence, process, or results used to reach a defensible conclusion. Use it when teams need to distinguish building the product right from building the right product.	conduct a design review; document the design review	Contrast: design review is a structured examination of evidence; it is not a conclusion reached before the evidence is checked. Example: The team will conduct a design review before it recommends a corrective action.
acceptance criteria	In medical devices, acceptance criteria is an approved rule or decision framework that sets what is permitted, required, or acceptable. Use it when teams need to distinguish building the product right from building the right product.	apply the acceptance criteria; comply with the acceptance criteria	Contrast: acceptance criteria is an approved rule or threshold, not an informal preference. Example: Please confirm which acceptance criteria applies before we commit the team.

Usability and Human Factors

Term	Working meaning	Collocations	Contrast and workplace line
human factors	In medical devices, human factors is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to explain use errors without blaming users.	clarify human factors; document human factors	Contrast: human factors should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify human factors and the evidence that supports it.
use error	In medical devices, use error is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to explain use errors without blaming users.	clarify use error; document use error	Contrast: use error should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify use error and the evidence that supports it.

Term	Working meaning	Collocations	Contrast and workplace line
formative study	In medical devices, formative study is a structured examination of evidence, process, or results used to reach a defensible conclusion. Use it when teams need to explain use errors without blaming users.	clarify formative study; document formative study	Contrast: formative study should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify formative study and the evidence that supports it.
summative study	In medical devices, summative study is a structured examination of evidence, process, or results used to reach a defensible conclusion. Use it when teams need to explain use errors without blaming users.	clarify summative study; document summative study	Contrast: summative study should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify summative study and the evidence that supports it.

Regulatory Pathways and Submissions

Term	Working meaning	Collocations	Contrast and workplace line
510(k)	US FDA premarket submission that typically demonstrates substantial equivalence to a legally marketed predicate device.	prepare a 510(k); identify a predicate device	Contrast: A 510(k) is a regulatory submission pathway, not a general approval for any future device use. Example: Regulatory will prepare the 510(k) around the intended use and predicate comparison.
De Novo	In medical devices, De Novo is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to use pathway language without promising clearance or approval.	clarify De Novo; document De Novo	Contrast: De Novo should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify De Novo and the evidence that supports it.
PMA	In medical devices, PMA is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to use pathway language without promising clearance or approval.	clarify PMA; document PMA	Contrast: PMA should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify PMA and the evidence that supports it.
predicate device	In medical devices, predicate device is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to use pathway language without promising clearance or approval.	clarify predicate device; document predicate device	Contrast: predicate device should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify predicate device and the evidence that supports it.

Complaints, MDRs, and Postmarket Signals

Term	Working meaning	Collocations	Contrast and workplace line
complaint	In medical devices, complaint is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to triage field reports with safety and regulatory discipline.	clarify complaint; document complaint	Contrast: complaint should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify complaint and the evidence that supports it.
MDR	In medical devices, MDR is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to triage field reports with safety and regulatory discipline.	clarify MDR; document MDR	Contrast: MDR should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify MDR and the evidence that supports it.

Term	Working meaning	Collocations	Contrast and workplace line
malfunction	In medical devices, malfunction is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to triage field reports with safety and regulatory discipline.	clarify malfunction; document malfunction	Contrast: malfunction should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify malfunction and the evidence that supports it.
postmarket surveillance	In medical devices, postmarket surveillance is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to triage field reports with safety and regulatory discipline.	clarify postmarket surveillance; document postmarket surveillance	Contrast: postmarket surveillance should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify postmarket surveillance and the evidence that supports it.

Manufacturing, Suppliers, and Nonconformance

Term	Working meaning	Collocations	Contrast and workplace line
nonconformance	Failure to meet a requirement, specification, contract, standard, or approved procedure.	clarify nonconformance; document nonconformance	Contrast: nonconformance should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify nonconformance and the evidence that supports it.
supplier CAPA	In medical devices, supplier CAPA is a stakeholder category whose needs, eligibility, experience, or obligations affect the decision. Use it when teams need to communicate product-quality problems without hiding supply risk.	clarify supplier CAPA; document supplier CAPA	Contrast: supplier CAPA should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify supplier CAPA and the evidence that supports it.
process validation	In medical devices, process validation is a repeatable control mechanism used to prevent, detect, or confirm conditions before work proceeds. Use it when teams need to communicate product-quality problems without hiding supply risk.	clarify process validation; document process validation	Contrast: process validation should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify process validation and the evidence that supports it.
lot traceability	Ability to follow a production lot through materials, process steps, tools, inspections, and downstream customers or products.	clarify lot traceability; document lot traceability	Contrast: lot traceability should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify lot traceability and the evidence that supports it.

Clinical Training and Labeling Boundaries

Term	Working meaning	Collocations	Contrast and workplace line
labeling	In medical devices, labeling is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to train users while staying inside cleared indications and instructions.	clarify labeling; document labeling	Contrast: labeling should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify labeling and the evidence that supports it.

Term	Working meaning	Collocations	Contrast and workplace line
instructions for use	In medical devices, instructions for use is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to train users while staying inside cleared indications and instructions.	clarify instructions for use; document instructions for use	Contrast: instructions for use should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify instructions for use and the evidence that supports it.
off-label	In medical devices, off-label is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to train users while staying inside cleared indications and instructions.	clarify off-label; document off-label	Contrast: off-label should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify off-label and the evidence that supports it.
clinical evidence	In medical devices, clinical evidence is information that can be checked to support a claim, decision, investigation, or compliance conclusion. Use it when teams need to train users while staying inside cleared indications and instructions.	clarify clinical evidence; document clinical evidence	Contrast: clinical evidence should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify clinical evidence and the evidence that supports it.

Industry-Specific Meeting Moves

Situation	Useful language
User Needs and Design Inputs	Before we commit, I want to confirm user need, design input, the owner, and the evidence behind the decision. If user need, design input, intended use, and risk must be separated., I recommend we document the risk and agree on the next step.
Risk Management and Hazard Analysis	Before we commit, I want to confirm hazard, harm, the owner, and the evidence behind the decision. If severity, probability, detectability, and mitigation evidence need review., I recommend we document the risk and agree on the next step.
Verification, Validation, and Design Review	Before we commit, I want to confirm verification, validation, the owner, and the evidence behind the decision. If validation, usability, clinical workflow, and traceability may still be open., I recommend we document the risk and agree on the next step.
Usability and Human Factors	Before we commit, I want to confirm human factors, use error, the owner, and the evidence behind the decision. If interface design, labeling, workflow, and risk controls need assessment., I recommend we document the risk and agree on the next step.
Regulatory Pathways and Submissions	Before we commit, I want to confirm 510(k), De Novo, the owner, and the evidence behind the decision. If predicate comparison, indications, performance data, and agency review remain uncertain., I recommend we document the risk and agree on the next step.
Complaints, MDRs, and Postmarket Signals	Before we commit, I want to confirm complaint, MDR, the owner, and the evidence behind the decision. If complaint intake, adverse-event assessment, and reporting timelines may apply., I recommend we document the risk and agree on the next step.
Manufacturing, Suppliers, and Nonconformance	Before we commit, I want to confirm nonconformance, supplier CAPA, the owner, and the evidence behind the decision. If nonconformance, disposition, supplier capa, and process validation must be resolved., I recommend we document the risk and agree on the next step.
Clinical Training and Labeling Boundaries	Before we commit, I want to confirm labeling, instructions for use, the owner, and the evidence behind the decision. If labeling, intended use, clinical evidence, and compliance boundaries control the response., I recommend we document the risk and agree on the next step.

High-pressure pushback frames

- I understand the urgency. The risk is that we move faster than the evidence or process supports.

- I am not blocking the goal. I am naming the condition we need before the decision is safe and credible.
- If we accept this risk, we should name the owner, document the assumption, and define the trigger for escalation.
- That may be possible, but not under the current scope, timeline, or approval path.
- Let's separate what we know, what we assume, and what still needs confirmation.

Contrast Pairs

Do not confuse	Useful distinction
user need vs traceability	In user needs and design inputs, define whether the discussion is about the current fact pattern, the controlling process, the stakeholder pressure, or the final decision.
hazard vs residual risk	In risk management and hazard analysis, define whether the discussion is about the current fact pattern, the controlling process, the stakeholder pressure, or the final decision.
verification vs acceptance criteria	In verification, validation, and design review, define whether the discussion is about the current fact pattern, the controlling process, the stakeholder pressure, or the final decision.
human factors vs summative study	In usability and human factors, define whether the discussion is about the current fact pattern, the controlling process, the stakeholder pressure, or the final decision.
510(k) vs predicate device	In regulatory pathways and submissions, define whether the discussion is about the current fact pattern, the controlling process, the stakeholder pressure, or the final decision.
complaint vs postmarket surveillance	In complaints, mdrs, and postmarket signals, define whether the discussion is about the current fact pattern, the controlling process, the stakeholder pressure, or the final decision.
nonconformance vs lot traceability	In manufacturing, suppliers, and nonconformance, define whether the discussion is about the current fact pattern, the controlling process, the stakeholder pressure, or the final decision.
labeling vs clinical evidence	In clinical training and labeling boundaries, define whether the discussion is about the current fact pattern, the controlling process, the stakeholder pressure, or the final decision.